

2026 Tepper Healthcare Case Competition

sponsored by Vertex Pharmaceuticals



Disclaimer

Although this case is based on an actual disease in Vertex's pipeline, the case material should not be considered a projection of any action taken or to be taken by Vertex Pharmaceuticals.

The following materials can only be used for participation in the 2026 Tepper Healthcare Case Competition.

Competition guidelines and deliverables

Qualifying Round | Submission Deadline: October 4, 2026 at 11:59 PM EST

Deliverable

- A 10–15-page PowerPoint slide deck draft outlining your findings and key recommendations (in PDF)
- The content should be the priority in this draft

Final Round | Revision Submission Deadline: October 20, 2026 at 11:59 PM EST

Deliverable

- A finalized PowerPoint slide deck providing a business case for your recommended approach, including appendices and support materials (in PDF). The main body of the deck should not be more than 20 slides.
- The final version of the deck should be ready to present to an Executive audience

If selected as a finalist, your team will have 20 minutes to present your findings and recommendations to the judging panel. Judges then will have 10 minutes to ask questions about your recommendations and thought processes. All team members are expected to be present during the presentation as the questions from judges might be directed to one specific individual or the whole team.

With that in mind, significant changes to the overall strategy and recommendations will not be allowed after finalist teams are announced.

Teams are encouraged to research external information and create baseline assumptions to use in their proposal. These assumptions should be documented. It's less critical what the actual baseline values are. Judges will be looking to see how teams are able to develop their solutions, justify their proposal, apply critical thinking, problem solve and apply creativity. There is not a one-size fits all answer.

The Company

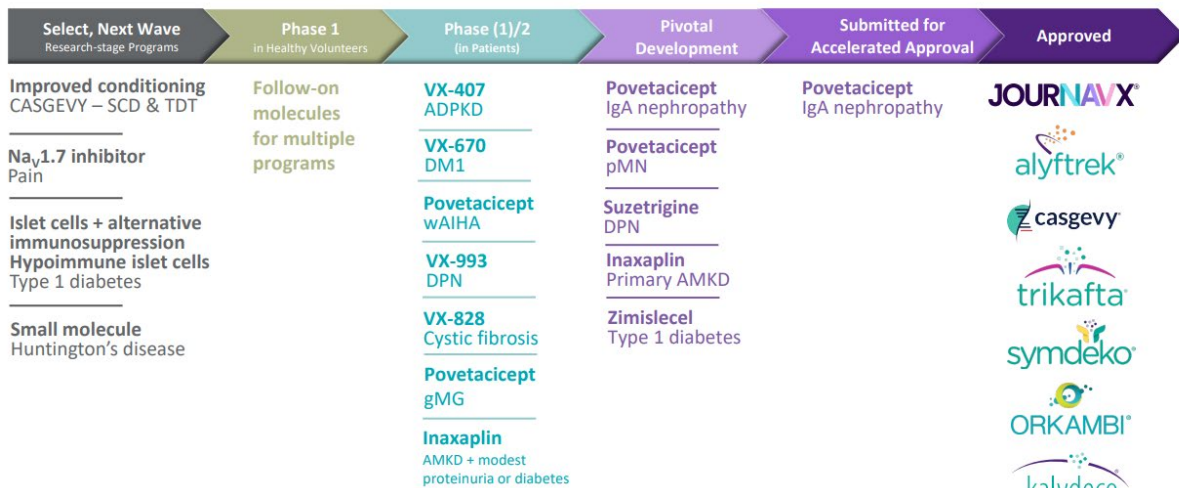
Vertex is a global biotechnology company that invests in scientific innovation to create transformative medicines for people with serious diseases. The company has multiple approved medicines for cystic fibrosis, sickle cell disease / beta thalassemia, and acute pain.

Beyond these approved products, Vertex has a robust clinical pipeline of investigational small molecules, biologics, and cell and genetic therapies in other serious diseases where it has deep insight into causal human biology, including IgA nephropathy (IgAN), diabetic peripheral neuropathy (DPN), APOL1-mediated kidney disease (AMKD), type 1 diabetes (T1D), and more.

For the purposes of this case competition, we would like participants to focus on Vertex's investigational AMKD program.

VERTEX R&D Portfolio:

R&D portfolio is broad, deep, and rapidly advancing



SCD: sickle cell disease; TDT: transfusion-dependent beta thalassemia; alt. IS: alternative immunosuppression; CF: cystic fibrosis; AMKD: APOL1-mediated kidney disease; ADPKD: autosomal dominant polycystic kidney disease; DPN: diabetic peripheral neuropathy; DM1: myotonic dystrophy type 1; pMN: primary membranous nephropathy; gMG: generalized myasthenia gravis; wAIHA: warm autoimmune hemolytic anemia.

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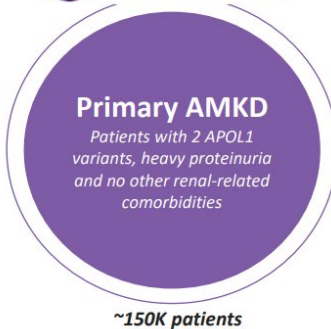
10

Source: Vertex Presentation, Q1 2026 Earnings Call, May 2026

AMKD program upcoming milestones

AMKD: Upcoming milestones for inaxaplin in APOL1-mediated kidney disease

First potential targeted therapy for patients with AMKD



- **IA data expected early 2027** (after 48 weeks of treatment); if positive, file for potential accelerated approval in the U.S.
- **On track to complete full study enrollment by YE:26**
- **IA endpoints:** eGFR slope vs placebo and percentage change from baseline in proteinuria versus placebo



~100K additional patients

- Phase 2 POC study of inaxaplin with **two patient cohorts not studied in AMPLITUDE:** AMKD with modest proteinuria and AMKD with moderate/severe proteinuria and diabetes
- On track to complete study and share **results H2:26**

9

Source: Vertex Presentation, Q1 2026 Earnings Call, May 2026

AMKD Program Background

Vertex Pharmaceuticals is preparing for the US launch of inaxaplin, a first-in-class oral therapy for APOL1-mediated kidney disease (AMKD). AMKD is a genetic condition that causes rapid kidney function decline primarily in people of African descent, specifically those who carry two APOL1 high-risk gene variants (G1/G1, G2/G2, or G1/G2). 13% of African Americans have the two high-risk gene variants, and 20% of those who have two high-risk variants will go on to develop AMKD.

There are no treatments today for AMKD specifically, and AMKD patients only have standard chronic kidney disease (CKD) treatments available to them (ACEs, ARBs, etc.). However, a combination of factors have resulted in AMKD being significantly underdiagnosed: 1) the genetic relevance of APOL1 in kidney disease is newly recognized 2) the majority of potential AMKD patients (not diagnosed patients) are currently managed in primary care setting and 3) AMKD diagnosis requires genetic testing, which is growing but limited in nephrology practices, and is largely not conducted in primary care setting.

Vertex has already built out a field sales force to commercialize poveticept for IgA nephropathy and now must consider what additional resources will be required across nephrology and primary care to get patients diagnosed with AMKD and treated with

inaxaplin (i.e. inaxaplin will be Vertex's second asset in nephrology). Vertex currently does not currently have a field sales force involved in primary care across any disease, and there are far more primary care physicians than there are nephrologists in the US.

The Strategic Challenge

You are part of Vertex's US commercial team tasked with developing the go-to-market strategy for inaxaplin. Your team must address two critical questions related to AMKD patients currently managed in primary care:

1. Primary care strategy

Many AMKD patients are managed in primary care settings and may not see nephrologists until late-stage disease. However, Vertex has no primary care sales force and limited brand presence outside specialty nephrology.

Please evaluate two options to expand our efforts to reach patients who are currently managed in primary care:

Option 1 (Sales): Adjusting or supplementing the existing nephrology sales force, which could take the form of any of the 3 following options:

- a. **Expansion:** Expansion of the sales force to include a dedicated FTE team to call on primary care physicians
- b. **Contracting:** Expansion of the sales force to include contracted supplemental sales force to call on primary care physicians
- c. **Retargeting:** Revising the target list of the existing nephrology sales force to include top primary care targets (and reducing focus on some nephrologists)

Option 2 (Marketing): Expansion of direct-to-patient advertising efforts to encourage genetic testing for APOL1 risk variants

You must recommend zero, one or both of the Sales and Marketing options available to you, though do not recommend multiple of the sub-options for Sales (e.g., if recommending Option 1, please specify 1a vs. 1b vs. 1c). Please make your recommendations specific with quantitative assumptions, including values for level of investment / headcount.

2. Financial evaluation of recommendations

Your recommendation must include specific quantitative estimates for required investments, and financial justification based on expected patient reach, genetic testing uplift, revenue impact, NPV, ROI and payback period. Clearly state your assumptions and financial risks or opportunities not included in the base case.

The Data

There is no explicit provided dataset for analysis in this year's case competition as the focus is intentionally on your strategic and financial evaluation of inaxaplin commercialization options.

However, we have provided equity analyst reports detailing Vertex and our kidney portfolio to assist you with constructing your case submissions and to give you a frame of reference as you determine your assumptions. Please utilize these reports as needed.

Evaluation Criteria

Case competition submissions will be judged on the following four criteria:

- **Strategic acumen:** Ability to critically evaluate the trade-offs across the sales and marketing options, select a defensible recommendation with clear rationale grounded in Vertex's existing capabilities, competitive positioning, and the unique challenges of treating AMKD patients currently in primary care settings
- **Financial analysis:** Quality of the quantitative model supporting the recommendation, the transparency and soundness of key assumptions, the credibility of projected financial returns, and the ability to identify material financial risks and opportunities not fully captured in the recommendation
- **Written communication:** Team's written submission presents a logically structured, concise, and professionally compelling argument that clearly connects the strategic recommendation to its financial justification and underlying assumptions
- **Verbal communication** (final presentation only): Ability to confidently articulate and defend their primary care strategy and financial model in a live setting, demonstrating command of their assumptions and adaptability under Q&A from the judging panel